Journal of Psychological Inquiry
2025, Vol. 29, No. 2, pp. 5-14
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Elizabeth A. Dahl, Ph.D.,
Best Paper Award

PREDICTORS OF SUSCEPTIBILITY TO FALSE ONLINE MENTAL HEALTH INFORMATION

MADDEN G. RITZER AND DONNA W. NELSON

WINTHROP UNIVERSITY

Abstract – We investigated factors that predict susceptibility to false online mental health information. We introduced a fictitious mental health disorder, fear of future anxiety (FOFA) and presented the information to young adults in different online formats and with differing prevalence rates. Contrary to our hypothesis, we did not find evidence supporting the idea that the format would alter beliefs about FOFA. Instead, we observed that young adults were susceptible to the false mental health information presented online, regardless of the format or prevalence rate, particularly among those who relied more heavily on social media for health information. Notably, young adults were more inclined to diagnose others with the false mental health issue than they were to diagnose themselves. Moreover, individuals more inclined to believe they had FOFA tended to be White, women, and non-heterosexual. Consistent with our hypothesis, they also displayed characteristics such as heightened sensitivity to criticism, less resilience, and increased feelings of imposter syndrome. These findings highlight young adults' receptiveness to mental health information online regardless of its validity and provide new insight into how unhealthy personality traits might enhance this receptiveness.

People frequently use online media as a source of health information, with an increasing number of people specifically seeking mental health information (Starvaggi et al., 2024). The use of online resources is particularly high for people in crisis situations and when mental health resources are not physically available (Cohen et al., 2023; Situmorang, 2020). Young adults, because of their high levels of interaction with social media in general, are frequent consumers of online mental health information (Pretorius et al., 2019).

Online mental health information can be very beneficial. For example, the availability of social media content related to mental health facilitates help-seeking and creates a sense of community for young people struggling with mental health issues (Li et al., 2023; Zhang et al., 2024). Struggling individuals can both offer and receive social support from others, or receive support from professional providers, online in a way that might not be possible in the physical world (Naslund et al., 2020). Online information also increases public awareness of disorders and has been shown to reduce the associated stigma (Herrera-Peco et al., 2023; Jankowski et al., 2023).

Although online mental health resources can be beneficial, they also have the potential to be harmful. Individuals struggling with a particular disorder can easily access communities that perpetuate the problem, such as pro-eating disorder sites (Zhang et al., 2024). In an examination of the usefulness of mental health communities, Li et al. (2023) discovered that the quality of help-offering replies was inconsistent and in part depended on how accurately the help-seeking adults were able to present their issue. An additional concern is that viewers can misuse information. After TikTok influencers described having unusual and atypical symptoms of rare mental health disorders, psychologists saw a growing number of young adults present with the same symptoms (Olvera et al., 2021). Similarly, people with some mental health issues, such as borderline personality, may exhibit false symptoms that they find online, compromising their ability to receive an accurate diagnosis and treatment (Bastiaens et al., 2024).

The accuracy of online mental health information varies (Starvaggi et al., 2024; Wilhelm et al., 2022). Online information is presented more frequently by non-expert users than by discipline-specific experts, raising concerns about its validity (Reed, 2023). Content moderators lack clear guidance about when and how to present mental health information, resulting in varied quality of what is approved (Hoops et al., 2023). Even when trained therapists use social media to discuss mental health, there are limited ethical guidelines and oversight (White & Hanley, 2023).

Consumers of online mental health content may not have the expertise to discern its quality for themselves. Heiss et al. (2024) presented adults with

short Tik Tok videos that varied in whether the narrator was an expert or not and whether the information was supported with scientific references. Both types of presenters similarly changed viewers' perceptions, and the use of scientific support had no impact. Twitter researchers found that retweeting posts about mental health research had more to do with whether the tweet contained a hyperlink than with the quality of the information (Madden et al., 2024). Ravi et al. (2022) recently argued that self-diagnostic symptoms provided on social media tend to be general enough that even healthy adults may be led to believe that they have a mental health issue.

To further investigate factors that predict belief in fake mental health information presented online, we created a false mental health disorder, fear of future anxiety (FOFA). This fake disorder was inspired by the rising anxiety rates in young adults in the United States, particularly in groups with instability issues such as being unmarried or lower SES (Goodwin et al., 2020). We presented information about the disorder to young adults online via a self-diagnostic test or in a scientific summary of symptoms. We also provided different estimates of its prevalence during early adulthood as being high (65%) or low (15%). We hypothesized that participants would be more likely to perceive the disorder as real, agree that people have it, feel sympathy for those individuals, and show interest in learning more about it when:

1. they were given the self-diagnostic test instead of the scientific summary, because of the personally relevant nature of the information, and simultaneously provided with a higher estimated prevalence rate, suggesting that the disorder is common in their age group,
2. they experienced the unhealthy psychological traits of: sensitivity to criticism, fear of missing out, imposter syndrome, and low resilience,
3. they reported higher levels of reliance on social media for health information.

We also conducted exploratory analyses to investigate whether demographic characteristics of our participants would predict their belief in the fake disorder.

Method

Participants

This study was approved by the university's Institutional Review Board and used a convenience sample. Participants were recruited through college classrooms, social media, and word-of-mouth. Potential participants were provided with a link to the online study, where they encountered additional information about the study and the informed consent. Each participant had to indicate, in the online system, that they agreed to participate before accessing the survey.

We had a total of 105 participants complete our survey; however, one did not correctly respond to the manipulation check item. Thus, the final sample consisted of 104 young adults with a mean age of 19.44 (*SD* = 2.46). Among them, 74 were women, 24 were men, and six categorized themselves as non-binary or "other." Regarding race, 57 participants were White, 30 were Black, and the remainder identified as Multiracial/Other (seven participants), Asian (five participants), or Latinx/Hispanic (four participants). In terms of sexual orientation, 61 participants were heterosexual, 23 were bisexual, eight were gay or lesbian, eight identified as "other," and four did not provide a response.

Materials and Procedure

First, participants responded to four demographic items to assess age, gender, race, and sexual orientation. All participants were then instructed, "You will be learning about the Fear of Future Anxiety (FOFA). You will then be asked questions to test your understanding of this disorder. Please read carefully." Participants then encountered information about FOFA that was designed to look and feel like online health resources, with concise descriptions, bulleted lists, and prevalence statistics. This approach was meant to reflect how online health information is usually encountered. Information varied in two ways: presentation format (scientific summary- see Appendix A or self-diagnostic test-see Appendix B) and prevalence rates (high-65% or low-15%) of the disorder in the college population. Participants were randomly assigned to one of four conditions, resulting in 27 participants in the scientific presentation/low prevalence condition, 27 participants in the diagnostic test/low prevalence condition, 26 participants in the scientific presentation/high prevalence condition, and 24 participants in the diagnostic test/high prevalence condition.

Participants were then presented with six statements describing symptoms to which they could rate their level of agreement on a 5-point scale where 1 represented "strongly disagree" and 5 represented "strongly agree." They were instructed, "The more you agree with these items, the more likely it is that you have symptoms of this disorder." Immediately following the FOFA information, participants encountered a manipulation check item that provided the correct definition of FOFA. Participants could respond "true" or "false." Participants had to respond correctly to be included in the study; the one participant who did not was excluded. Next, participants responded to four researcher-created items to assess 1) interest level in learning more about FOFA, 2) belief that they personally

had the disorder, 3) belief that someone they knew had the disorder, and 4) level of sympathy for people who have the disorder. Responses were made on a 5-point scale where 1 represented "strongly disagree" and 5 represented "strongly agree."

Participants also indicated how often they relied on online resources to learn about mental health issues and how often they believe others are to do this. The response options for both of these items ranged from 1 representing "never" to 5 representing "all the time." We then asked participants the percentage of college-aged students who experience symptoms of this disorder. The experimental conditions had indicated that prevalence rates were either 15% or 65%. The response options were 15%, 25%, 50%, 65%, and 80%.

Next participants responded to four previously published scales. On all four of these scales, there were five response options with 1 representing "strongly disagree" and 5 representing "strongly agree." The first scale included four items comprising the Behavioral Engagement sub-scale and two items from the Affective Engagement subscale of the *Social Media Engagement Scale* (Ni et al., 2020). The published reliabilities for this scale are .71 to .81 (Ni et al., 2020), and we achieved a reliability of .84. Participants then responded to the 6-item *Brief Resilience Scale* (Smith et al., 2009), The published reliabilities for this scale are .80 to .91 (Smith et al., 2009), and we achieved a reliability of .85. Participants responded to the 9-item F*ear of Missing Out Scale* (FOMO; Przybylski et al., 2013). The published reliabilities for this scale are .87 to .90 (Przybylski et al., 2013), and we achieved a reliability of .82. Participants responded to the 7-item *Leary Imposterism Scale* (Leary et al., 2000). The published reliability for this scale is .95 (Freeman et al., 2022), and we achieved a reliability of .93. Last, participants responded to was the 3-item *Sensitivity to Criticism Scale* (Rosenberg, 1965). Responses were made on a 5-point scale where 1 represented "almost never" and 5 represented "almost always" We achieved a reliability of .86.

All participants were debriefed at the end of the study that FOFA was a fictitious disorder and to the purpose of the study.

Results

Hypothesis 1: Comparison of Experimental Condition

A 2 (information presentation) X 2 (prevalence) MANOVA revealed no interactions or main effects among the four conditions on our dependent variables: interest in learning about FOFA, belief participants knew someone with the disorder, belief they personally had the disorder, and sympathy level for people with the disorder. See Table 1 for means and standard deviations.

Although these were not hypothesized, we found two significant main effects, both relating to participants' beliefs about the prevalence rates of the disorder. Participants who were provided high prevalence rates reported a higher prevalence rate (M = 2.19, SD = 1.51) than participants who were provided low prevalence rates (M = 3.92, SD = .71), [$F(1, 99) = 57.65$, $p < .001$]. We also found that participants who took the diagnostic quiz (M = 3.33, SD = 1.44) reported higher prevalence rates than those who read the scientific summary (M = 2.68, SD = 1.45), [$F(1, 99) = 8.12$, $p = .005$].

Hypothesis 2: Trait Predictors of Susceptibility

We examined relations among variables using Pearson's correlations and the data can be seen in Table 2. Participants who agreed more that they personally had FOFA were more sensitive to criticism, had lower resilience, had higher imposter syndrome, and had higher FOMO. These traits did not predict participants' agreement that they knew someone with FOFA. Participants who felt sympathy for those with FOFA had lower resilience. These represent weak to moderate correlations.

Table 1
Means (and Standard Deviations) for Dependent Variables Across Experimental Conditions

	Scientific Summary	Self-Diagnostic Test	Prevalence Low	Prevalence High
I believe that I might have this disorder.	3.38(1.07)	3.06(1.35)	3.30(1.14)	3.12(1.32)
I believe that I know someone with this disorder.	3.80(.99)	3.57(1.45)	3.85(1.22)	3.50(1.26)
I am interested in learning more about FOFA.	3.86(.93)	3.55(1.15)	3.81(1.04)	3.58(1.07)
I have great sympathy for those with this disorder.	4.14(.86)	4.10(.99)	4.23(.95)	4.00(.88)

Table 2
Correlation Matrix for Traits, Social Media Reliance, and FOFA Perceptions

	1	2	3	4	5	6	7	8	9	10
1 Resilience	-									
2 Sensitivity to criticism	-.31**	-								
3 FOMO	-.21*	.54**	-							
4 Imposter syndrome	-.23*	.43**	.49**	-						
5 Reliance on social media	-.09	.17	.25*	.22*	-					
6 Use social media for mental health info	-.20*	.22*	.30**	.08	.15	-				
7 Have FOFA	-.36**	.28**	.39**	.24*	.15	.24*	-			
8 Know someone with FOFA	-.07	-.01	.14	-.04	.08	.08	.56**	-		
9 Have sympathy for FOFA	-.25*	.05	.05	.08	.02	.26**	.48**	.44**	-	
10 Want to learn about FOFA	-.26**	.08	.04	-.06	.19	.25*	.41**	.21*	.28**	-

Note. * $p < .05$. ** $p < .01$.

Hypothesis 3: Reliance on Social Media

Participants who relied more on online mental health resources also agreed more that they personally had FOFA, were interested in learning about FOFA, and felt sympathy for people with FOFA (see Table 2). These are weak correlations. General reliance on social media did not significantly predict any of these variables related to FOFA.

Participants who agreed more that they rely on online resources to learn about mental health issues, had: lower resilience, higher sensitivity to criticism, and higher FOMO. General reliance on social media was predicted by imposter syndrome and FOMO. These are weak to moderate correlations.

Exploratory Analysis: Demographic Predictors

Next, we examined demographic categories as predictors of beliefs about FOFA and online mental health information using independent *t*-tests. Means and standard deviations can be seen in Table 3 and independent *t*-test data can be seen in Table 4. We

Table 3
Means (and Standard Deviations) for Dependent Variables Across Demographic Categories

	Hetero-sexual	Non-hetero-sexual	Black Adults	White Adults	Men	Women
I believe that I might have this disorder.	3.07 (1.26)	3.53* (1.13)	2.86 (1.22)	3.33* (1.28)	2.61 (1.16)	3.38** (1.22)
I believe that I know someone with this disorder.	3.64 (1.23)	3.82 (1.23)	3.59 (1.20)	3.80 (1.22)	3.13 (1.33)	3.86** (1.19)
I am interested in learning more about FOFA.	3.68 (.99)	3.74 (1.18)	3.66 (1.23)	3.65 (.97)	3.26 (1.25)	3.83* (.98)
I have great sympathy for those with this disorder.	4.05 (.92)	4.24 (.94)	4.21 (.98)	4.05 (.91)	3.83 (.94)	4.19* (.91)
I often use online resources to learn more about mental health issues.	3.51 (1.17)	3.97* (.75)	3.83 (.89)	3.62 (1.18)	3.26 (1.25)	3.75* (1.00)
Reliance on social media	3.69 (.74)	3.54 (.99)	3.39 (.95)	3.72 (.84)	3.14 (.88)	3.73** (.82)
People are too quick to believe mental health information they encounter online.	4.20* (.74)	3.87 (.07)	4.21 (.77)	4.02 (.97)	3.57 (1.04)	4.31** (.71)

Note. * $p \leq .05$. ** $p < .01$.

Table 4
Independent t-test Statistics for Comparisons Across Demographic Categories

				95% CI		
	df	***t*-value**	***p***	**Lower**	**Upper**	**Cohen's *d***
SEXUAL ORIENTATON						
Have FOFA	95	-1.82	.036	-0.96	0.04	1.21
Use social media for mental health info	95	-2.18	.016	-0.89	-0.04	1.03
People are too quick to believe mental health information they encounter online.	95	1.83	.036	0.03	0.70	0.88
RACE						
FOFA	82	-1.61	.053	0.34	0.77	1.26
GENDER						
Have FOFA	93	-2.66	.005	-1.34	-0.20	1.20
Know someone with FOFA	93	-2.49	.007	-1.31	-0.15	1.22
Want to learn about FOFA	93	-2.28	.013	-1.07	-.07	1.05
Have sympathy for FOFA	93	-1.67	.049	-0.81	0.07	0.92
Reliance on social media	93	-2.91	.002	-0.98	-0.18	0.84
Use social media for mental health info	93	-1.91	.029	-1.00	0.02	1.07
People are too quick to believe mental health information they encounter online.	93	-3.88	<.001	-1.12	-0.36	.80

compared Black adults to non-Hispanic White adults, our only two racial groups with sufficiently sized samples, and found that compared to Black adults, White adults agreed more that they had FOFA.

Compared to heterosexual adults, non-heterosexual adults were more likely to agree that they had FOFA, more likely to rely on online mental health resources, and less likely to agree that people are "too quick to believe mental health information they encounter online."

Compared to men, women were more likely to agree that they had FOFA, knew someone with FOFA, were interested in learning about FOFA, and felt sympathy for people with FOFA. Women also had higher social media reliance scores than did men, relied more on online mental health resources, and agreed more that people are "too quick to believe mental health information they encounter online."

Discussion

Hypothesis 1: Comparison of Experimental Conditions

We hypothesized that participants would be more susceptible to believing the fictitious disorder of FOFA if they took a self-diagnostic test after having been told the disorder had high prevalence. We failed to find support for this prediction. Regardless of the way the information was presented, participants seemed to believe that FOFA existed. For example, they indicated great sympathy for those with the disorder.

Participants believed there were higher prevalence rates when they were told the disorder had a high prevalence rate. This outcome is logical and expected. Additionally, we found that participants who took the self-diagnostic test picked higher prevalence rates than participants who encountered the scientific

summary about FOFA. In other words, when the information was presented in a personally relevant way, people assumed that the disorder was more common, even if they were told otherwise. This outcome was unexpected but might be explained by the false-consensus effect, where people make the cognitive mistake of assuming others share their beliefs and behaviors (Ross et al., 1977). Engaging with the self-diagnostic test might have led participants to identify with the disorder's characteristics, making them more likely to believe that others experience it too.

Hypothesis 2: Trait Predictors of Susceptibility

Providing some support for our hypothesis, the susceptibility to false online mental health information co-occurred, although weakly, with unhealthy psychological traits. Young adults who perceived themselves to have FOFA were more sensitive to criticism, were less resilient, and had a higher FOMO. Similarly, young adults who relied more on online resources for mental health information were more sensitive to criticism, were less resilient, and had higher FOMO; these findings support the previously established link between social media use and higher rates of mental health concerns (Nazari et al., 2023; Plackett et al., 2023). These relationships were weak, which suggests there is much to learn. There are likely other variables that better predict susceptibility to false online information, such as the demographic predictors found in our study. In addition, young adults' receptiveness to online mental health information might be so ubiquitous that it is not necessarily limited to those who are psychologically unhealthy.

Hypothesis 3: Reliance on Social Media

Participants who relied more on online resources to learn about mental health were also more likely to believe they had FOFO, feel sympathy for people with FOFA, and want to learn more about FOFA. These outcomes were specifically tied to using social media for mental health information, because general reliance on social media did not predict participants' belief about FOFA. This outcome provides mixed support for our hypothesis that adults who reported higher levels of reliance on social media would also be more likely to believe in the existence of FOFA. Instead, it seems that individuals who specifically trust the credibility of online mental health information are more susceptible to believing false or exaggerated claims about mental health conditions.

Imposter syndrome was the one psychological trait that predicted susceptibility to false online information but not a reliance on online resources for mental health information. One likely explanation is that imposter syndrome is characterized by fear, particularly fear of one's success being uncovered as fraudulent at some point in the future (Clance & Imes, 1978). This fear of an anxiety-provoking future event overlaps with the symptomology of FOFA, all of which focus on the fear of feeling anxiety in the future. However, imposter syndrome may not influence reliance on online mental health resources because its core concern is self-doubt and perceived fraudulence rather than a general tendency to seek or trust online information.

Exploratory Analysis: Demographic Predictors

Although we saw susceptibility to the false online information in the sample as a whole, young adults who were non-heterosexual, White, and women were more likely to believe that they personally had FOFA. Women and non-heterosexual adults also turned to online resources more for their mental health information than men and heterosexual adults. This pattern of receptiveness matches previous research showing that women are more likely than men to utilize, and have positive attitudes toward, online mental health programs (McCall et al., 2023; Smail-Crevier et al., 2019), while Black adults have historically harbored more negative perceptions of mental illness and treatment than White adults (Harris et al., 2020). Less information is known about non-heterosexual adults' engagement with online mental health information; however, Lucassen et al. (2018) reported that LGBTQ+ individuals were receptive to online information, especially in contemporary and engaging formats.

Compared to men, women also were more likely to agree that they knew someone with FOFA, felt sympathy for people with the disorder, and were interested in learning more. These responses may reflect women's socially-driven tendency to report higher levels of empathetic emotions (Pang et al., 2023). Another possibility is that women were more responsive because the information was presented online. The women in our study reported a greater, general reliance on social media than did men, and it is possible that the reliance on social media led them to trust the online content more.

Limitations

This study has limitations worthy of consideration. First, our sample was predominantly composed of college-aged, White women, and the use of a convenience sample further limited the demographic diversity of our participants. Another potential limitation of our study concerns the manner in which participants encountered information about FOFA. While we tried to use a presentation format similar to what individuals might encounter while searching online for health information, it is worth noting that encountering

information within the context of a scientific study may differ qualitatively with respect to credibility and trustworthiness, compared to online searches. It is possible that participants generally tend to trust information embedded in research studies, which may have dampened any potential differences that alternate formatting may have elicited. Additionally, we did not directly verify whether our materials truly mirrored what participants would typically see in their online searches. Finally, while we used symptoms that were designed to align with the characteristics of FOFA, they may have been broad enough that participants found it easy to agree with them, which could have influenced their responses. Future research could consider exploring the effects of presenting more unique or unconventional symptoms to assess whether they would yield different results.

Future Directions

There are several additional areas that could benefit from further investigation. Future research could investigate whether beliefs based on misinformation persist over time and whether repeated engagement with false health information strengthens its impact. Future studies could also examine how media literacy and the ability to critically evaluate content affect how people respond to online misinformation or test the impact of information formatting on the behaviors and attitudes of individuals engaging in real-world online searches of mental health information. Finally, future research could explore ways to help people become more skeptical of faulty online mental health information. Educational initiatives could help to mitigate the spread of misinformation and improve ability to identify reliable sources that support a more informed approach to mental health.

Conclusion

In sum, we found that young adults were susceptible to the false mental health information regardless of the format or purported prevalence rates, especially young adults who relied more heavily on social media for health information or were White, women, or non-heterosexual. This susceptibility to online mental health information is concerning because not all information online is credible (Starvaggi et al., 2024; Wilhelm et al., 2022). The problem may become self-perpetuating, because young adults who rely heavily on online sources of mental health information were also the ones who expressed interest in learning more and felt the most sympathy for people with FOFA. In other words, they may utilize low quality resources to gain information and then have an increased desire to engage further, both cognitively and emotionally, with the faulty information.

Our findings highlight the urgent need to address this issue through initiatives aimed at ensuring that trustworthy mental health information is both readily available and easily accessible. Additionally, efforts should focus on helping young adults develop stronger skills to critically evaluate online content. This will empower them to recognize credible sources and make informed decisions about their mental health.

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Author Note

Correspondence concerning this manuscript should be addressed to Donna W. Nelson, Department of Psychology, Winthrop University, Rock Hill, SC 29732.

Email: nelsond@winthrop.edu.

Appendix A

Experimental Condition: Scientific Summary of Fear of Future Anxiety

Researchers have recently identified a new disorder called Fear of Future Anxiety (FOFA). The main symptom is worry that overwhelming anxiety will occur sometime in the near future.

This disorder is most common in young adults. Estimates are that 65% (or 15%) of college-aged people experience symptoms.

In order to be diagnosed, the individual must experience:

- Fear of future anxiety that causes life disruption for at least four weeks
- Worry and fear about having anxiety in the future that the individual can't control
- Avoidance of thinking about the future out of a fear that those thoughts will create anxiety

This disorder has been shown to drastically interfere with daily life activities for those who have it.

Appendix B

Experimental Condition: Diagnostic Test of Fear of Future Anxiety

Below is a tool to assess symptoms of Fear of Future Anxiety (FOFA). This disorder is most common in young adults. Estimates are that 65% (or 15%) of college-aged people experience symptoms.

The more you agree with these items, the more likely it is that you have symptoms of this disorder.

I often spend time worrying that something is about to trigger my anxiety.

I am not anxious at the moment but I fear that I could become anxious at any time.

I avoid thinking about the future because I worry that it will make me feel anxious.

I am fearful of how the future might make me feel anxious.

I avoid things that I think might trigger my anxiety about the future.

I worry that the future is going to be filled with anxiety.